SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Fluorouracil + folinic acid every 14 days

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Adjuvant or advanced colorectal cancer · NCCP 00660 · version 2b · 14-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Treatment after colorectal cancer surgery can reduce recurrence risk. For advanced cancer, treatment aims to control the disease. Your team confirms which aim applies.

Schedule
Day 1: folinic acid and fluorouracil injection, followed by a 46-hour fluorouracil pump. Repeat every 14 days. Adjuvant treatment: up to 12 cycles; advanced treatment: continue only while appropriate.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Folinic acid
CHEMOTHERAPY MODULATOR

- ☐ Enhances fluorouracil activity and toxicity
- ☐ Rare allergic reaction
- ☐ Diarrhoea / mouth soreness in combination

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity

Confirm treatment aim, expected benefit, reasonable alternatives and no-treatment/supportive-care options. Discuss DPD, infection, mouth/bowel/skin toxicity, cardiac symptoms, fertility, contraception, breastfeeding and interactions. Explain line/pump risks and response to malfunction.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00660 v2b.

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